



MANAGING CONSUMERS' HEALTH IN THE HEAT AND SMOG

Introduction: As the temperature rises, and the smog level increases, there is the potential for serious health concerns that staff must be aware of and address. This inservice will attempt to cover some of these concerns and point out antecedent and follow-up actions that should be taken to maintain the health and safety of our consumers.

General Concerns:

As the temperature rises, people's comfort-level becomes challenged. The smog level increases with the heat and this adds to the discomfort. **Staff attention must be paid to providing adequate cool water (not soda) intake, checking for weather appropriate clothing, protecting skin from the sun, not overdoing the activity level, and assessing the comfort needs of each consumer.**

- **Providing adequate fluid intake involves providing sufficient opportunities for the consumers to drink fluids.** Staff should provide two pints of cool water per consumer per day, more if necessary. (A typical bottle of drinking water is a pint.) Water can be purchased with ADS program money and stored in a cooler with ice or freezer packs in the Job Coach's car and carried in bottles in the backpack, and drinking fountains can be used. Additionally, consumers should be encouraged to eat breakfast and drink fluids first thing in the morning. The Job Coach should provide opportunities to drink additional fluids throughout the day.
- **Checking for whether appropriate clothing involves inspecting the dress of the consumers as they join the program and making corrections before leaving their home.** People should wear clothing that is light in color and weight, allow for ventilation, and cover the skin to prevent sunburn. Hats should be encouraged and sunglasses allowed.
- **Protecting the skin from the sun involves adequate covering and the use of sunscreen.** Clothing/hats are the simplest protection. Sunscreen may be purchased with program money and should bear a code number higher than 15. It should be applied before the consumers go into the sun.
- **Activity choices should be directed toward shady outdoor settings, and indoor ones (particularly with air conditioning).** Heavy work-related activities should be scheduled first thing in the morning before it gets hot. Work alternatives can be politely asked for that involve less heavy work or indoor work. An activity involving using an air-conditioned setting should be planned for daily. This can include the library, bowling alleys, stores and malls, movie theaters, restaurants, etc.
- **Assessing the comfort needs of each consumer involves asking about and observing them for comfort related issues.** If a person appears affected by the weather,



they should be given less demands, opportunities to rest, extra fluids, or whatever appears reasonable. They should be touched to determine if they are hot or feverish and observed for lethargy or sleepiness. They may require medical evaluation and the ADS office should be notified (see below).

Medical Concerns: People challenged with seizures, psychiatric issues, high blood pressure and other diagnoses can react to the heat and smog much more severely than other persons. Staff must be aware of the medical issues associated with their consumers and of the specific issues the heat and smog may raise in their care.

- **Many medications cause "photosensitivity"** which involves making the person taking the medication susceptible to sunburn and rash with even moderate exposure to the sun. Some medications taken by many of our consumers with this side effect include: Tegretol, Seroquel, and Risperdal. Staff will look for this information in the "side effects" section of each ADS report. Management of this problem includes covering the skin with clothing/hats and use of sunscreen.
- **Persons diagnosed with seizures have a higher likelihood for experiencing seizures as the heat/smog level rises.** They should not be overheated by exertion or overexposure to the sun. They should be allowed to sit out, work, spend time in air-conditioned settings and watch for signs of seizure activity. Any report from the consumer, or direct observation, that they are not feeling/acting normal should be responded to with a call to the ADS office. They may be sent home or, in extreme circumstances, be taken by ambulance to an emergency room.
- **Persons taking the medicine Lithium Carbonate are taking salt and should be encouraged to consume adequate fluids.** Failure to do so can result in toxic symptoms. These include diarrhea, vomiting, tremor, mild ataxia, drowsiness, or muscular weakness. Giddiness, ataxia, blurred vision, tinnitus (ringing in ears) and a large output of dilute urine are signs of higher-level toxicity. A physician should be notified if any of these symptoms are observed.
- **Persons with excessive perspiration or no perspiration at all, a fever, feeling weak, acting sleepy, etc. may be reacting to the heat/smog.** Any deviation from their acting their "usual selves" should be looked for. The ADS office should be called by the Job Coach to discuss the observed symptoms. The consumer may be given light duty, taken home, or taken to an emergency room depending upon the conditions.

Please consider that we will not be criticized for acting too conservatively in protecting the health of our consumers. If you suspect a problem, do what you need to solve it before it becomes worse.



Arroyo

Developmental Services

A Near Tragedy

In the summer of 2005, one of our groups in Pasadena had a consumer who we have served for many years that became so severely affected by the heat that he nearly died. Even after our efforts to provide water and fluids, to seek low demand activities and shade, to use an air-conditioned car and keep a good watch over him, he went unconscious within blocks of his drop-off from apparent heat stroke. He was immediately taken by ambulance to a nearby hospital after being placed in the Intensive Care Unit for several days until he made a full recovery. There were lessons learned from this near tragedy that need to be considered by all ADS caregivers.

- This consumer was on a lot of medications that caused him to be sedated, sleepy, have poor balance (ataxia) and appear “droopy” even on cool days. This medication interfered with his ability to perspire normally, to be alert enough to drink and eat normally and masked the symptoms of heat related problems. Staff must be extra careful with consumers who are taking a lot of medications and are very sedated on a regular basis. They are most likely to be affected by the heat. The ADS office must be made aware of this problem in order to look into the appropriateness of serving these individuals on very hot days.
- This group had someone else in it who was intolerant of many community settings with people in it and would holler loudly, tantrum and become aggressive in these settings. Consequently the group spent their days visiting various parks and open settings. On hot days they were sitting in the shade, but still in the heat and humidity. Efforts were needed to seek out air conditioning for some of the activities. Libraries, stores/malls, bowling alleys, restaurants, and an ADS office were all possibilities. Swimming pools are open as are gyms in large parks. Without the opportunity to really cool off (remember, their bedrooms often do not have air conditioning), the body’s temperature can gradually rise to a fever and proceed to heat stroke over a period of days.
- The consumers must be strongly encouraged to drink water (not soda) throughout the day. The cooler the fluids are kept, the better. The consumers should be observed to see that they are perspiring normally, they should be touched to see if they are running a fever, they should be talked to often to see if they are alert and how they are feeling. If they are not perspiring, are hot to the touch, or are lethargic and “spacey” the ADS office must be notified. Staff may be instructed to cool them down with water, take them to the ADS office, to their home, or even call “911”.
- Staff will routinely offer to “wet down” consumers when outside for activities. They may wet their hair, hat, shirts, arms and legs, etc., in an effort to promote keeping cool. Normal perspiration is impaired by medications people are taking so getting wet is the next best thing.

ADS caregivers must make every effort to plan for the heat of the season when serving our consumers. Staff “common sense”, initiative and prompt intervention can save lives. The ADS administration will do everything possible to facilitate this process.



POLICIES FOR SUPPORTING CONSUMERS DURING HIGHER TEMPERATURES

As the maximum daily temperature exceeds 85-90 degrees (usually in May-June) the Executive Director and the Program Director will identify the at risk consumers for heat and smog issues. These will be consumers with high levels of medications that can affect the body's ability to deal with the heat, people with partially controlled seizures, and people who are morbidly obese, elderly or otherwise medically fragile. High levels of medications will include two or more Neuroleptics, mood elevators, blood pressure medications and/or seizure medications. These people are often photosensitive, falling asleep during part of the program day and may already have some restrictions on their participation in general activities. The identified consumer's activity schedule will be scrutinized and plans made for adequate opportunities to rest, drink adequate water, and visit settings with air conditioning for a minimum of three hours a day. Their direct care staff will receive supplemental training on supporting them during the heat, their group compositions may be adjusted to permit extra support and their health status monitored more closely. Additional advice from their physicians may be requested to determine how best to serve them during higher temperatures.

During this period of higher temperatures, the overall day program expectations will shift to ensure the safety and comfort of all program participants. This will be started with the "Heat and Smog" Inservice. All groups will be expected to modify their activities to take account of the heat and are expected to spend at least one major activity a day in air-conditioned settings. This lasts typically two hours. They will also be expected to cool down their cars before loading passengers into them. They will offer water (not soda) frequently to consumers, provide hats and sunscreen, and know the symptoms of heat exhaustion and heat stroke and how to respond. During routine monthly inspections held by ADS administrators (the Program Service Review) the vehicles will be inspected for the presence of working air-conditioning, adequate cool water (at least one quart per person per day), window shades, seat covers when necessary. Staff will be quizzed on their preparedness to deal with heat related issues.



Expressive and Receptive Language Issues with the Intellectually Disabled

People with an intellectual disability often have difficulties with:

- processing and learning new information,
- following directions,
- understanding and using written and spoken language (e.g. unable to follow conversation or verbally communicate their thoughts to others),
- comprehending detailed information
- completing tasks.

A person with an intellectual disability and communication deficits may also **struggle with social skills and understanding the non-verbal communication of others**, such as understanding facial expression, gestures or social etiquette. In people who do not speak at all, or have limited verbal abilities, they can be helped by developing alternative communication methods, such as using assistive technology such as picture boards.

Helping People with Speech Issues

As with most therapeutic interventions, the first step generally involves obtaining a complete background of the person's social, education and medical history, as well as their current level of functioning in regard to speech and communication.

Speech therapy and receptive language: Receptive language refers to the person's ability to take in and understand information. We can help a person with receptive language challenges by working on areas such as:

- building eye contact,
- using minimal instructions and basic language,
- expanding the individual's vocabulary,
- focusing on specific words to learn,
- incorporating visual cues such as pictures, gestures or body language to enhance understanding.

Support for issues involving receptive language often involves asking the person to state what has been said to them and helping the person recognize an instruction or concept and act on the information.

Speech therapy and expressive language: Expressive language involves the ability to express ideas, choices, feelings and needs. Hardships in this area can be improved by:

- constant engagement in conversation, such as asking them to describe their ideas or actions,
- modeling the mechanism of generating speech sounds and words to the person,
- using books to tell stories and encouraging letter writing,
- broaden the person's vocabulary, which is often done by exposing them to social experiences, reading or consuming other media, or engaging in exercises where they have to name objects or actions in everyday activities.

Speech therapy and social language: Social or practical communication skills are required to properly adopt language within a social context. These skills are often crucial for a person to engage in their community and with their peers, and involve:

- How we use language in various ways (e.g. making requests of other people, social greetings);
- Adapting how we use language to the social situation (e.g. we often speak differently to our peers than we do to a stranger or a person older than us);
- Learning social etiquette and standards (e.g. understanding social cues, importance of taking turns).

We can enhance practical language skills by applying aids (e.g. the use of photos that help us say what we want to say) and modeling appropriate conduct in specific social contexts (e.g. greetings, manners), using role playing and teaching the individual to better read and understand facial expressions. It is also critical for the person to be engaged in a variety of social interactions in everyday life.

Speech therapy and persons who do not use speech: When a person is showing signs of being unable to produce sounds or speak, it becomes necessary to find alternative ways of communication. This often includes the use of gestures and body language, visual aids and objects (e.g. picture boards or cards), or learning sign language. Therapy usually has an initial focus of developing the person's skills to be able to communicate practical messages, such as how to show that they are hungry and represent emotions (e.g. using picture cards to display if they are tired or happy). It may take some time to identify what is most comfortable for the person to use in terms of alternative communication approaches. As things progress and the person's communication skills increase, we will work to enhance the person's ability to manage complex tasks and instructions that include more actions.

Speech Therapy Games for Adults

Speech therapy activities are a fun way for people of all ages to develop the skills they need to communicate better and gain confidence.

1. **Heads Up! Smartphone Game:** Heads Up! is a smartphone app game that combines trivia and charades. (download it from the Apple App Store or from Google Play.) The

player who will be guessing the word or phrase holds the smartphone on their forehead, revealing the word or phrase to others, who act out clues. Helps with: Learning to speak in turn.

2. **Opposites Attract:** In this game, one player makes a list of words that have a clear opposite, such as "yes," "down," "happy," etc. The player reads each word aloud and the other player states the opposite. Helps with: Word finding, speaking in turn.
3. **Group Chat:** Set a topic, such as a movie or current event, and create a safe space for players to discuss. This is a great way to practice more natural communication. Helps with: Speaking in turn, sentence fluency.
4. **Name Game:** This activity involves naming items within a certain category. The best way to play is with cards that have written directions on them, such as "Name five fruits." In turn, each player picks a card and responds to the directions. Helps with: Word finding.
5. **Journaling:** Because reading and language development are interconnected, writing in a journal can help people improve language production and comprehension. Provide a list of writing prompts that the player can choose from. Have them write without stopping for 5 or 10 minutes. Helps with: Word finding, sentence fluency.
6. **Name Ten:** Similar to the name game, the player is given a category and is asked to name ten things that fit within that category. For example, you can ask the player to name ten colors, fruits, animals, or sports. Helps with: Word finding.
7. **Sound-Focused Game:** Players choose a sound they want to target, such as an "S" or "T" sound. Then they take turns saying sentences aloud using as many words as possible that include that sound. For example, if players are targeting an "S" sound, one player might say, "My sister Sally sat sideways in the seat." Helps with: Pronunciation, word finding.

Activities to work on speech, language and communication goals.

- **Newspapers.** A newspaper—target literacy, answering and asking questions about current events, searching for a movie time and location, and social skills or abstract language in the comics section.
- **Magazines.** Age-appropriate and interesting magazines contain a variety of articles, pictures, advertisements and more. Also, the magazines your clients choose may give you insights into their interests and motivations.
- **Grocery circulars.** Use free circulars to learn money management, categories (such as food groups) and new food vocabulary.

- **Brochures/catalogs.** Brochures and catalogs motivate and engage clients depending on their interests. If your client likes electronics, bring an electronics catalog, for example. Discuss prices, various types of equipment and what they like versus dislike. The same approach works for clothing, gardening or home décor catalogs.
- **Subway/bus maps.** Work on travel training, literacy and map reading with these resources. Language concepts include problem-solving, sequencing and answering "wh" (who, what, where, when and why) questions.
- **Menus.** What's more functional than being able to read a menu and make a choice?
- **Employment applications.** Stop into any fast-food establishment, restaurant, movie theater or retail store and ask for an application. Filling out an application facilitates improved literacy, answering "wh" questions, recalling information, expanding vocabulary, and sequencing by writing the order of educational or work history.
- **Dominoes.** A set of dominoes offers an inexpensive, portable, age-appropriate and fun developmental activity for adults. Practice matching, taking turns, solving problems and following directions.
- **Playing cards.** Ideas for card games include Uno, Go Fish and War. Examples of targeted goals include matching, solving problems, taking turns and prediction.
- **Board games/bingo.** How about a game of Sorry or Connect Four? These games teach taking turns, learning colors, following directions and solving problems.



Establishing Rule Governed Behavior

In Applied Behavior Analysis (ABA), establishing rule-governed behavior means teaching a client to follow verbal or written instructions, known as "rules," instead of relying on direct experience with consequences. This ability is crucial for safety and independence, as it allows individuals to learn complex or dangerous skills without having to experience the outcome firsthand.

A key concept is the distinction between

-governed & contingency-shaped behavior.

Feature	Rule-Governed Behavior	Contingency-Shaped Behavior
Control	Behavior is controlled by verbal or written rules, such as "If you want to stay safe, then look both ways before you cross the street".	Behavior is shaped and controlled by direct, immediate consequences in the environment.
Learning Process	Individuals learn by following instructions and don't need to directly experience the outcome. A person who has never gotten a speeding ticket still drives the speed limit because of the rule.	Learning occurs through trial and error, where the consequences of an action directly affect future behavior. A person who has gotten a speeding ticket before may now always drive the speed limit.
Key Advantage	Allows for rapid learning of complex skills and avoidance of dangerous consequences.	The resulting behavior is highly sensitive to changes in the environment, as it is based on real-time feedback.

Several strategies to help clients develop rule-following skills.

Break down instructions

Break complex tasks into small, manageable steps. A rule like "Clean your room" can be broken into "Pick up toys," "Put clothes in the hamper," and "Make the bed". When making a rule regarding inappropriate conduct, state the rule using positive terminology (example "sit like a lady with your hands to yourself" rather than "no touching others" or "Help the coach stay safe when driving" rather than "don't touch the coach when they are driving"). The simpler the person, the more direct the rule.

Use visual supports

Visual learners benefit from tools that make rules more concrete and understandable.

- **Visual schedules** lay out the steps of a routine, like getting ready in the morning.
- **Contingency maps** visually represent "if-then" relationships and the consequences of following or not following a rule. This is the Premack Principle.
- **Social stories** use pictures and simple language to explain social situations and rules.

Practice with reinforcement and modeling

- **Reinforce success:** Offer praise or rewards when the client successfully follows a rule to increase motivation. This can be a token economy system or specific praise like, "Great job waiting your turn!".
- **Model behavior:** Therapists and caregivers can model the desired behavior to show the client how to follow the rule in practice.
- **Fade prompts:** Therapists may initially use prompts to help initiate the behavior, then gradually fade them as the client becomes more independent.

Generalize across environments

Use natural environment teaching (NET) to ensure that skills learned in one setting carry over to other settings, like at work, in the car, while in the community.

Challenges and considerations

- **Rule insensitivity:** An overreliance on rules can sometimes make an individual insensitive to changing contingencies, following an outdated rule even when it's no longer effective.
- **Complexity:** Some individuals, particularly those with more limited verbal skills, may not acquire rule-governed behavior easily, and specific procedures may need to be tailored to their needs.
- **Consistency:** For rules to be effective, they must be delivered with clear and consistent language. Without consistent follow-through, the client may learn that instructions don't always need to be followed.

A variety of challenges can complicate the establishment of rule-governed behavior, especially for individuals with conditions like autism spectrum disorder (ASD) or certain behavioral disorders. These challenges include verbal and cognitive difficulties, competing environmental contingencies, motivational issues, and the risk of rule-based insensitivity.

Cognitive and verbal challenges

Rule-governed behavior requires a certain level of verbal and cognitive skill, and deficits in these areas can hinder the process.

- **Difficulty understanding abstract rules:** A rule is a verbal description of an action and its consequences. If a learner struggles with language comprehension, they may not grasp the abstract nature of a rule, such as "be polite". Instead, they may require more concrete, specific instructions, like "say 'excuse me'".
- **Prompt dependency:** Some learners may become overly dependent on prompts and fail to develop independent rule-following. A prompt-dependent learner might wait for a cue before following a rule, even if they have the skill to do it on their own.
- **Problems with verbal rehearsal:** Many people silently repeat rules to themselves to guide their behavior. Individuals who do not engage in this self-talk may have a harder time using rules to manage their actions.

Insensitivity to changing contingencies

One of the most significant challenges is that overreliance on rules can cause a learner to become insensitive to the actual consequences in the environment.

- **Rigidity and inflexibility:** When rules dictate behavior, a person might become so rigid that they have difficulty adapting to a change in circumstances. For example, a student who is told to "always raise your hand" might not speak up during a group activity, even when the teacher has given the group a different, more flexible instruction.
- **Psychological inflexibility:** A strong tendency to rigidly adhere to internal, self-imposed rules can be linked to psychological suffering. For example, a person with a fear of gaining weight might follow rigid dietary rules despite the severe negative health effects.

Competing contingencies

A learner may fail to follow a rule if other, more powerful environmental contingencies push them toward a different behavior.

- **Conflict between long-term and immediate consequences:** A rule often describes a consequence that is delayed or improbable ("studying now will lead to a good grade later"). However, the immediate reward of engaging in a preferred, rule-breaking activity (like playing video games) can be a more powerful motivator.
- **Social reinforcement:** For many people, the approval or disapproval of others is a powerful motivator. A learner might break a rule to gain peer acceptance, even if they know the rule is important.

Motivational and contextual factors

Several other variables can influence how well a person responds to rules.

- **Emotional regulation issues:** For individuals with challenges in emotional regulation, overwhelming emotions like anxiety, frustration, or excitement can make it difficult to follow instructions.
- **Lack of belief in the rule:** A person is more likely to follow a rule if they have a history of reinforcement for following instructions and perceive the person giving the rule as reliable. If they have had experiences where rules were inconsistent or unreliable, they may not believe the described contingency will actually happen.
- **Environmental inconsistency:** A lack of consistency in rule enforcement across settings or individuals can make it difficult for a person to learn rule-governed behavior. For example, if a child is told not to run inside at school but is allowed to do so at home, they may become confused.



SPECIAL INCIDENT REPORT PREPARATION

by Robert Wark

The Special Incident Report is a critical document to be used by staff to describe the circumstances surrounding unusual occurrences that involve the consumers of our agency. This document is always reviewed internally by agency administrators and often sent to outside regulatory agents.

When are special incident reports written?

Special incident reports recount events that are outside of the normal routine of the consumer's life. They are prepared by the specific staff involved in the incident and are completed as soon as possible after the incident. They may describe an injury, an act of aggression, property destruction, or the requirement of emergency procedures of restraint employed by the staff. This includes any incident witnessed by, or involving "third party" participants such as family members, community members, police or other people (who may choose to report their version of the incident). Observed or suspected violation of anyone's rights is documented in an incident report. Circumstances which affect the ability of a consumer to function normally; including an observed change in their normal behavior pattern is reported on an incident report.

How are special incident reports written?

The report begins with basic information regarding which program the incident occurred in, who it involved (including witnesses), the date of the report, the date, time and specific location of the incident and what "type" it was. The "types" of incidents offered provide further information as to what constitutes an incident that requires reporting.

Completing the "description of incident" section involves the careful depiction of what happened before the incident that may or may not be related to it, a complete description of the incident itself, including how staff may or may not have involved themselves, and how the incident ended. The report may exceed the space provided on the form by continuing on a separate sheet.

The incident is reported objectively, without the inclusion of personal feelings and interpretations (nothing about who was "angry" at who or how "upset" someone became). It details what was seen, heard and actions that were taken. Descriptive words are used that avoid "jargon". "Tantrum", "aggressive", or "self-injurious" (etc.) are avoided and replaced with descriptions of actions (like "Mike hit his head with his right hand approximately thirty times", or "Bob hit Sue with an open hand on her arm once; no tissue damage"). Descriptions of staff actions also avoid "jargon", such as "time out", "proning", "restrained", etc. Instead specific actions are defined ("Sue told Bob "stop" which was followed by giving him space and time until Bob was calm; 2 minutes")

Events are reported in chronological order and duration of specific events is estimated



on every incident report. Staff should indicate what happened first, second, third, etc., instead of reporting the "meat" of the incident only, or leaving out a part. Particularly important, staff do not refer to incidents in the past that are not themselves documented within the current report or in a previous report. The duration of episodes of aggression, outbursts, elopement, etc. are a primary method for indicating the intensity of incidents. Similarly, the duration of physical containment of any kind, used by staff, partially indicates the severity of the incident.

The remainder of the report involving immediate and preventative action taken is filled in by agency administrators (see below). The author of the description of the incident writes their name, title and the date in the box on the bottom of the second page.

What is done after the description of the incident is written?

Most special incidents require that an agency administrator is contacted at their conclusion in order to inquire as to what to do next. The administrator will expect an incident report to be written and submitted to them within 12 hours of the incident. They will read the report and may ask for additional information and even a rewrite of parts of the report in order to maximize its clarity. They may telephone outside agencies (as by law they are required to do, in specific instances) and will indicate who they spoke to on the incident report. Immediate action taken as follow up to the incident is reported by the administrator and its results. This may involve the addition of staff, the referral to medical resources, or other measures seen as appropriate to address the incident.

Agency administrators also write the "preventative action taken and/or planned" section. This is where the soundness of the staff intervention during the incident may be addressed. The implementation of changes in the consumer's environment may be described in order to limit future similar incidents.

The administrator then approves the report by writing their name, title and date in the correct box and it is distributed as indicated on the report.



SUPERVISION, SUPPORT AND PARTICIPANT AUTONOMY

1. Job Coaches will provide supervision and support to their clients in community settings during day program service hours. Supervision means that you can see them and get to them quickly to offer support (instruction and/or assistance) and/or keep them safe.
2. Clients will be supported to be as independent as possible, in their handling of their personal needs, in their day-to-day decision making, and in the activities of community living. They will be encouraged to develop agency (the feeling of control over their own actions and their consequences) and self-determination (providing direction over their own lives).
3. Clients may briefly leave the Job Coach's visual line of sight, to use the restroom, explore a store aisle, work in a different area of a jobsite, or make a purchase on their own. Each client's Individual Service Plan offers guidelines on how much autonomy they can safely handle. Some capable individuals can handle doing many things unsupervised; others must be kept by your side as much as possible. Clients will be instructed to discuss with their Job Coach where they will be, how long they will be "out of sight", and when and where they will rejoin the group. This is similar to the courtesy of friends who are visiting a setting together, who chose to separate and then rejoin each other. Clients will be thanked for informing the Job Coach of their plans and congratulated for following through on what they said they would do.
4. Job Coaches agree that they are "on duty" during lunch and two 10-minute rest periods, and are paid for that period of time. Job Coaches are encouraged to rest, eat and use the restroom whenever participants do so. Sharing breaks, snack and meal times, is conducive to conversation and social support. Working on client Data Packets can be accomplished when clients are relaxing in a safe setting (like a library) and can be monitored while the Job Coach makes their notes.
5. Clients may not be left in a parked car while the Job Coach enters a building. The exception may be at pick-ups/drop-offs, when a participant may be safer staying seated for several minutes while the Job Coach escorts other participants to/from the car. Lower several car windows during warm weather to ensure adequate ventilation is maintained in the cabin when the engine is off. Never leave your car keys in the ignition, or leave the car without the parking brake on while a client sits in your parked car.



6. Job Coaches will provide privacy to participants using a public restroom, but be nearby to offer support. They can be either inside the restroom, or standing outside by the door. Job Coaches should make themselves aware of who else is inside the restroom. Whenever possible, encourage the client to wait until community members (especially children) have finished using the restroom before going in themselves. If the Job Coach must use the restroom while clients wait, the Job Coach should ensure that they are seated comfortably nearby, with directions to remain seated and wait for the Job Coach's return. Individuals who are at high risk of wandering off, elopement or bothering others will be asked to wait inside the restroom with the Job Coach.
7. Job Coaches are encouraged to program alongside another Coach as a means of providing additional supervision and support to their clients. Such pairing up must be approved by the Program Director, considering if the abilities and needs of all clients involved are similar enough so that they all may take part in the same activities and may or may not last all program day depending upon the Program Director's recommendations.
8. Job Coaches will discuss with the Program Director any concerns they have regarding supervision issues, including client requests for more autonomy or difficulty maintaining line-of-sight supervision of clients while in the community.



TRANSITIONS

It's not uncommon for some of our consumers to have trouble transitioning from one setting to another, from one task to another, and/or from one staff to another. Conduct issues ranging from anxiety to refusals to agitation to aggression to AWOL can result from this issue. In extreme forms people have refused to move for hours and some have shown extreme fight/flight reactions and have become a danger to themselves and others. One result of the failure to transition is that people may be unable to access services, support and experiences that they would enjoy and which could greatly benefit them.

Arroyo has been referred people where their transition issues are the principal reason for a day program with behavioral support. Once this issue is successfully addressed, the person often showed remarkable progress and enjoyed our program.

Who are the people with transition issues?

People too often are not considered when plans are made, when changes in those plans occur, and when routines are broken. Staff may not realize that the program participant has had bad previous experiences in certain settings. Staff may not adjust plans when program participants don't feel well. Too often people are just rushed, bullied, uninformed and denied the right to say "no!", "not right this minute", or even "what's in it for me!" Another big issue is that there is no plan or tolerance for when people fail to transition at first.

Some people are predisposed to transition issues. They include folks with the following diagnoses and/or conditions:

DIAGNOSIS/CONDITION	TRANSITION ISSUE
Autistic Spectrum Disorder	Strong preference for routine and difficulty handling novel and changing circumstances. They may also be sensitive to some environmental features (lights, sound, waiting in line, crowds, even smell)

Down syndrome	Strong preference for familiar people and routines and difficulty handling novel and changing circumstances. They can refuse to move for hours (including dropping weight)
Obsessive/Compulsive disorder	They may not transition until their routines are completed, checked and possibly repeated
Thought Disorders (particularly with paranoid issues)	A sense of not being safe, of not trusting others, of being overwhelmed by changes
Mood Disorders (particularly depressive)	They may withdraw, refuse to participate, refuse to move
Epilepsy (after big seizures)	They may be exhausted, disoriented and even agitated following big seizures
Heavily Sedated People	They may fall asleep, stagger when walking and have no interest in programming

CAUSES AND TREATMENTS FOR DIFFICULTY WITH TRANSITIONS:

General Causes:

CAUSE	TREATMENT
Previous bad experiences in similar circumstances	Learn history to determine what worked and failed in the past. Avoid recreating past failures by being sensitive to preference and signs of intolerance
Rigid expectations for sameness	Try to keep many things the same while changing only a few things at a time. Allow for observation of others first, permit partial participation, provide escape options, quit while people are still successful, and provide heavy incentives. Plan for an option should there be a failure to transition
Intolerance for the unfamiliar	Make them familiar first with pictures, stories, allow them to see first, lower demands and expectations, leave early, allow refusal
Sensitivity to crowds, lights, noise	Avoid the worst of it; provide adaptations (head phones, sun glasses, etc.).
A successful strategy for taking control, getting attention, and/or extracting concessions	Know history, avoid power struggle, give up control, provide incentives, withhold big attention, wait it out, stimulus change, (have someone else try to move them) recognize and plan for failure to transition.

Fears of being harmed, lack of trust, acute anxiety	Build trust, address the reality of the fears, and teach relaxation and desensitization
A strong dislike of a program that moves from place to place and uses community settings	Limit the number and type of transitions in their program and build tolerance over time

Morning Transitions:

CAUSE	TREATMENT
Fatigue, illness	Work with home staff to promote enough rest, adjust pickup time, and allow for partial participation. Insist on nonparticipation at times (consult Director)
Being rushed, treated rudely, or confrontationally	Try to calm things down, lower expectations; provide incentives, report abusive staff, build trust and friendships
Poor relationships with home staff and/or family.	See above. Try to provide support to staff by working with them (not for them).
Lack of support for attending the program.	Build support for the program or report issues to the Arroyo Director and inform Regional Center caseworker.
Competing contingencies.	Try to make staying home less attractive
Avoiding getting ready in the AM (bathing, grooming, eating).	Schedule bathing at night, finish grooming in DP, provide cheap breakfast.
Stress in AM.	Eliminate it as much as possible

Setting to Setting Transitions:

CAUSE	TREATMENT
They don't want to leave something they like.	Plan ahead in AM, communicate plans, provide incentives, be patient, avoid settings where always happens, try stimulus change (someone else inviting them to leave).
They don't know where they are going next.	See above, provide enhanced schedule (words, pictures, symbols). Have them involved in planning and revisions.
They don't like where they are going next.	Make what follows the worst activity the best activity.

They are sensitive to a break in routine.	Apologize, slow down transition, allow for partial participation, and heavily reinforce any tolerance.
They refuse to enter the car.	Try to calm things down by slowing the expectation for transition, build rapport first, and provide incentives (they can sit where they want, choose the radio station, etc.). Keep things very calm and encouraging when they try to move toward the car. Use child safety locks for safety in case they try to exit a moving car and don't travel far. Never place an agitated person in your car as they may damage it and Arroyo will not pay for the damage. Call the office if they fail to transition to get support.
They refuse to exit the car:	Place the car out of the sun, open the windows and have everyone exit the car who is willing (take your keys). Move the group a short distance away from the person and wait. Do not look at or discuss the situation with the person in the car and give them a lot of attention when they do leave.

Staff to Staff transitions:

CAUSE	TREATMENT
They are very attached to their former staff person.	Respect this and avoid needless changes, have them meet others while with favorite staff who also treat them well, explain the need for change and be calm and patient.
They never met the new staff person.	The new staff must be extra nice, use reinforcement more and better, try to follow old routines to limit other changes, and give the relationship time to build.
They were not prepared in advance for the change.	See above
They know and dislike the new staff and/or their consumers.	Respect opinions, seek resolution of grievances



RIGHTS OF THE INTELLECTUALLY DISABLED

1. A right to day program and home services that help you learn new and familiar things and make your life interesting. You should be given services with the right amount of support to match your needs.
2. A right to being treated with respect, to have your needs kept private from the public and to not be treated roughly.
3. A right to participate in programs and services that are paid for by the state, no matter what your support needs are.
4. A right to prompt medical care and treatment.
5. A right to religious freedom and practice.
6. A right to be with others and to go into the community to do things that you need and would enjoy.
7. A right to physical exercise and recreational activities.
8. A right to be free from being hurt, held down, tied up, locked in a room, given too much medication, or left without someone to help you when you need it.
9. A right to be free from dangerous activities/treatment.

If you feel you are being denied any of the above rights, please tell someone you trust to help you. You may also call your Service Coordinator at ELARC at (626) 299-4700.

This information was read/explained to the client by:



PERSONAL COMMUNITY

John O'Brien & Beth Mount. Make a difference: A guidebook for person-centered direct support. inclusion.com.

As you continue to support people to live fulfilling lives within their communities, it is essential to identify what experiences and accomplishments will increase their sense of freedom, membership, and belonging. In this session you will be introduced to the construct of the **Five Valued Experiences**. The experiences highlighted will support others in developing a sense of belonging, emphasizing each person's unique talents and capabilities.

All citizens have better life outcomes, and everyone's world grows more interesting, when communities offer rich opportunities for people to have these five valued experiences:

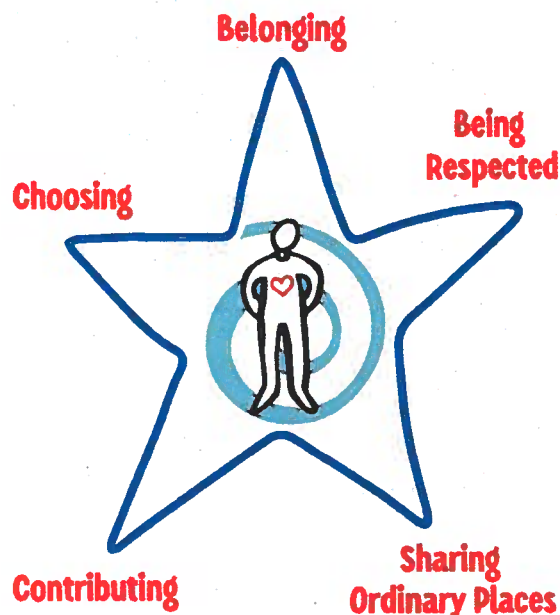
Belonging in a diverse variety of relationships and memberships.

Being respected as whole persons, whose history, capacities and futures are worthy of attention and whose gifts engage them in valued social roles.

Sharing ordinary places and activities with other citizens, neighbors, classmates, and co-workers. Living, working, learning, and playing confidently in ordinary community settings.

Contributing by discovering, developing, and giving their gifts and investing their capacities and energy in pursuits that make a positive difference to other people. There are gifts of being and gifts of doing: contributions can include interested presence as well as capable performance. Contributions may be freely exchanged (volunteered) or earn pay.

Choosing what they want in everyday situations in ways that reflect their highest purpose. Having the freedom, support, information, and assistance to make the same choices as others of a similar age and learning to make wiser choices over time. Being encouraged to use and strengthen their voice regardless of mode of communication, clarify what really matters, make thoughtful decisions, and learn from experiences.



GIFTS AND CAPACITIES

Explore gifts, skills and capacities, what a person is good at and what qualities the person has to contribute. Think about situations where the person can make a contribution to others and identify next steps.

Invite the person to talk about a time when something they did made a positive difference for others and gave the person a "good feeling." Build on this with stories from others who know them well about when the person is at their best. Consider what this tells us about the person's gifts, skills, and contributions.

Here is a construct that defines different gifts and capacities:

Gifts of the hand (and feet and voice) are abilities and skills that a person can contribute:

- dancing, singing, acting.
- typing, drawing, arranging shapes and colors, decorating
- using a computer.
- composing music, sewing.
- working hard, using physical strength.
- ability to sign, ability to speak or understand another language.

Gifts of the head include the knowledge, questions, experience, and information that a person can contribute:

- figuring people out, organizing and classifying things.
- travel routes, sports news, fan information, gossip.
- local history, trivia.

- creative thinking, math, solving puzzles.
- interest in politics, desire to think and talk about big questions

Gifts of history and identity include the experiences, knowledge, duties, responsibility concerns, types of belonging that come with membership in a family, religion, national or ethnic group, citizenship, or member of a club:

- Taking care of others as the oldest sibling
- Faith and Community Service

Gifts of the heart are the interests, enthusiasms, personal passions, and the rewards of relationship that someone brings to others:

- welcoming people, patience, courage, impatience to move from talk into action.
- sensitivity to others, high energy, desire for order, ability to listen.
- passion for justice, love of nature, ability to draw others out, spiritual gifts.

PLACES AND POSSIBILITIES

Identify the places that matter in a person's life. This increases understanding and helps identify places that are worth paying more attention to. Identify all the places a person goes that are important to them. Explore the role that the person has in each place. Think about places where the person's gifts and capacities might be welcomed and valued. Explore how this information can help in strengthening existing connections or where new connections can be made. Ask:

- Where is the person a ***customer?***
- Where is the person a ***member?***
- Where does the person ***feel at their best?***

Use the answers to these questions to expand the community of the person by asking:

- Where else might the person's gifts and capacities be ***welcomed?***
- Where might existing connections be ***strengthened?***
- Where might ***new connections*** be made?

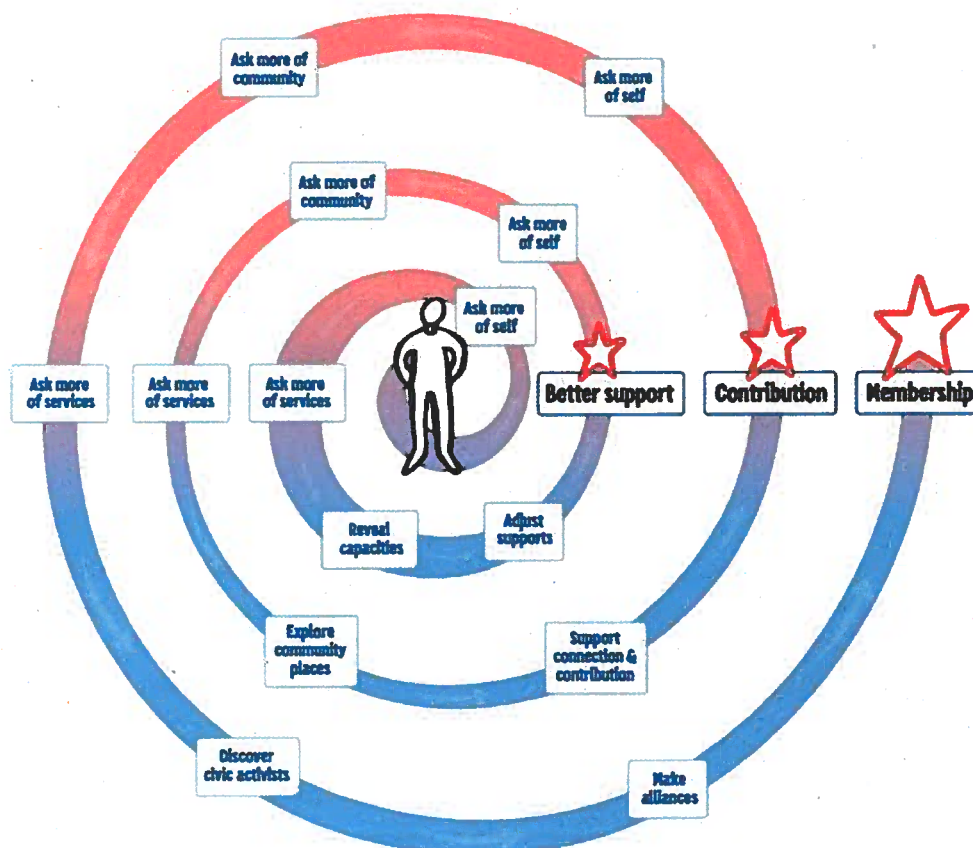
WHAT MORE IS POSSIBLE?

The "What More Is Possible" spiral, developed by John O'Brien and Beth Mount, is a journey of discovering new opportunities and possibilities for people, especially those with intellectual disabilities, to live a meaningful life. It centers on finding better ways to support people to live within their community, ensuring they get the support they truly want and need.

We do this by asking ourselves; what more do I want to ask of myself and my allies and how might I make a positive impact in people's lives, as we move along the journey to

reach belonging and membership. Imagine this journey as a spiral, starting at the center:

- We begin by getting to know the person on a deeper level, focusing on uncovering their skills and interests. This involves reflection on our own part and evaluating the support we offer, which ultimately reveals the person's capabilities and passions. Once these are identified, we act on what we learned and adjust our supports, thus leading to better supports and services.
- As we journey to the second spiral; we ask ourselves to go beyond just personal support. It focuses on helping a person be part of their community in a meaningful way. Not only asking more of ourselves, the supports that we offer, but of our community. Thus, creating new opportunities to explore where community connections may exist. Adjusting our supports to foster connection and contribution.
- The third spiral goes even further. It's about thinking bigger and asking, "How can I make a positive impact on my whole community?" This means looking at the community as a whole, its strengths, and problems, and finding ways for people with disabilities to make meaningful contributions and partnerships within the community. The spiral shows us that there are different levels of opportunity and change. Each level asks more of the person and the support they have. It also asks more of the community to be inclusive and provide opportunities for everyone.



Valued Experience	Share a time when this valued experience was present in your life. How did you feel?	How do you feel when this experience is not present in your life?
Belonging 		
Being Respected 		
Contributing 		
Choosing 		
Sharing Ordinary Places 		

PLACES AND POSSIBILITIES



Places where my contributions can **thrive**.

Identify the places and people that may welcome and value the person's gifts and contributions. Where am I a customer? What are the places where I feel good? What are the places where I am a contributor?

Where is the person
a customer?

Where is the
person a member?

Where does the
person feel at their
best?

Where else might
the person's gifts
and capacities be
welcomed?

Possibilities to explore.

What possibilities are there to strengthen existing connections?

What possibilities are there to make new connections?

What is the person drawn to? What does the person want to try?

What's next?

What will you do? Who are the people that can help?



Arroyo
Developmental Services

BLOODBORNE PATHOGENS

Bloodborne Pathogens They're bacteria or viruses that are present in the human blood and are disease-producing or infectious microorganisms that can be transmitted by means of blood, tissue, and body fluids containing blood such as saliva, and other bodily fluids. They are not limited to hepatitis B, hepatitis C, and human immunodeficiency virus (HIV). They can cause disease or infection in humans, especially in workers who are exposed to human bodily fluids. Such exposure involves but is not limited to: changing and disposing of diapers, cleaning up toileting accidents, assisting in menstrual care, and applying first aid.

Watch the 7 minute Video! <https://youtu.be/8B7VxfUNgkg?feature=shared>

Protect Yourself!

- Wear gloves when contact with bodily fluids is necessary
- Assume that contact with all blood or other body fluids is hazardous
- Wash anything that may have been exposed: wash hands/ skin or flush mouth and eyes
- Wrap all materials contaminated by blood (bandages, menstrual pads) in toilet paper or paper towels and dispose of responsibly
- Menstrual pads will be placed in the receptacle found in the stall of the women's bathroom
- Clean contaminated surfaces with disinfectant cleaner
- If working with a consumer with **Hepatitis B (HBV)** get a **vaccination**

If you think you have been exposed:

- Contact your supervisor and report the incident
- Fill out the confidential medical evaluation form provided by your supervisor
- See a doctor
- ADS provides HIV/HBV testing, at agency's expense, to all employees with a documented exposure incident

During all phases of Post Exposure, your confidentiality will be maintained on a "need to know basis". ADS provides the results of any HIV/HBV tests conducted to the exposed within 5 business days of receipt.

Universal Precautions

Simple infection control measures that reduce the risk of transmitting pathogens among caregivers and clients



Standard Precautions

1. Hand washing: with proper technique (longer/harder/hotter!), antibacterial soap, attention to nails/in-between fingers
2. Use of hand sanitizer (like Purell)
3. Avoid direct contact with body fluids of others (blood, mucus, saliva, etc.)

Contact Precautions

1. Direct contact: skin to skin – use gloves
2. Indirect contact: object to skin – use household disinfectants (Lysol spray/wipes) on frequently soiled or frequently touched surfaces

Airborne Precautions

1. At greatest risk within 3 feet or less
2. Transmitted by: coughing, sneezing, breathing, talking
3. Promote use of: "Dracula Cough", tissues, frequent hand washing

In Review: Practice Common Sense

Hand washing

Disinfect environment

Pay attention to those who are ill

Stay at home when you are sick

Build your immunity (eat/rest well, avoid habits that expose you to germs)

Available in the **OFFICE SHARED FOLDER**

4.1 Bloodborne Pathogen Diseases

4.1.1 Human Immunodeficiency Virus (HIV) - HIV is a virus that is incorporated into the genetic material of human cells and also reproduces within the cells. HIV cannot cross intact skin or mucous membranes, but must have direct access to white blood cells of the circulation system in order to establish an infection. More accurately, HIV infects the white blood cells, which are essential components of the body's immune system. Aids, which has no known cure, may result from HIV exposure. The concentration of HIV is highest in blood and semen; therefore, the virus may be transmitted parenterally or sexually. However, evidence strongly suggests that HIV cannot be contracted through casual contact.

4.1.2 Hepatitis B Virus (HBV) – There are approximately 300,000 cases of Hepatitis B in the United States every year. As many as one in four of these cases require hospitalization; while as many as



one in ten become carriers of the disease. Hepatitis B Virus causes Hepatitis, which is inflammation of the liver. The result may include a variety of symptoms and medical problems. Because Hepatitis B is able to cross the intact mucous membranes such as those of the eyes and mouth and is present in most body fluids, it is a much stronger virus than HIV and is able to spread very rapidly. In addition, the Hepatitis B virus may be transmitted either sexually or parenterally. All employees who have been identified as being exposed to potentially infectious materials will be offered the Hepatitis B vaccine at no cost. In addition, any employee who declines the vaccination must sign a waiver stating such.

4.1.3 Other Bloodborne Diseases - There are several other infectious diseases which are characterized by a phase in which the positive agent is circulated through the blood. These diseases are uncommon in the United States with the exception of syphilis and malaria.

4.2 Exposure Evaluation

4.2.1 Anytime an employee is exposed to a potentially infectious material, the employee shall immediately report the exposure to their supervisor. The supervisor shall provide the exposed employee with a confidential medical evaluation form. The supervisor will refer the employee to the agency's designated medical facility for a confidential medical evaluation. The medical evaluation includes the following:

4.3 Exposure Determination

4.3.1 The employees listed below are occupationally exposed to blood and other potentially infectious materials during clean-up and working with people with developmental disabilities served by the agency. Such exposure involves but is not limited to changing and disposing of diapers, cleaning up toileting accidents, assisting in menstrual care, and applying first aid.

4.4 Universal Precautions / Controls Measures

4.4.1 Universal precautions are observed in order to prevent employee contact with blood and other potentially infectious materials. Because bloodborne pathogens cannot be seen or otherwise detected, all employees will assume that contact with all blood or other body fluids is hazardous.

4.4.2 Hand washing facilities are readily accessible to all employees in community restrooms. Likewise, employees are required to wash their hands and all exposed areas of skin if they believe they have been exposed to bloodborne pathogens. In addition, mucous membranes that are exposed should be thoroughly flushed with plenty of water as soon as possible. If the eyes are exposed, they should be flushed with an eyewash solution as soon as possible.

4.4.3 All materials that have been contaminated by blood (menstrual pads) will be wrapped in toilet paper and disposed of in the receptacle found in the stall of the women's bathroom.

4.4.4 Good housekeeping practices and procedures are maintained in all areas. Equipment and



working surfaces are cleaned and decontaminated upon coming in contact with potentially infectious materials. Work surfaces are cleaned with an appropriate disinfectant, and all materials that are not decontaminated are placed in containers that are:

4.4.4.1 Secure.

4.5 Hepatitis B vaccination

All potentially exposed employees will be given the opportunity to receive a hepatitis B vaccination free of charge. Vaccinations are offered at the completion of the initial training program. Employees refusing to accept the vaccination will be required to sign the waiver form attached as appendix A.

4.6 Post Exposure Treatment and Notification Procedures

4.6.1 Should an employee or any employee acting as a "Good Samaritan" have an exposure incident, the employee is required to report the exposure to their supervisor who will advise the Program Director. The Program Director is responsible for documenting all occupational exposure incidents and reporting to the ADS Executive Director. Employees who are involved in an occupational exposure incident are requested to seek treatment and consultation with the agency's designated medical care provider.

4.6.2 ADS provides HIV/HBV testing, at the agency's expense, to all employees with a documented exposure incident. Should an initial blood test result be negative, the employee is retested at 6 weeks, 12 weeks and 6 months to determine if transmission has occurred. During this period, the employee should follow the recommendations provided by the treating health care provider.

4.6.3 Following the report of exposure, the agency will contact the exposure source and request that person be tested for HIV/HBV at the agency's expense. In the event the source individual is an employee and the employee refuses to be tested, such refusal will not affect that employee's future employment.

4.6.4 During all phases of Post Exposure, the confidentiality of the affected employee and exposure source will be maintained on a "need to know basis". ADS provides the results of any HIV/HBV tests conducted to the exposed and source employees within 5 business days of receipt.

4.7 Universal Precautions and General Safety Rules

4.7.1 **Universal Precautions** is an approach to infection control that classifies all human blood and certain human body fluids as infectious for HIV, HBV, and other bloodborne pathogens.

4.7.2 The following **Universal Precautions** and General Safety Rules have been established to prevent the spread of viral and bacterial organisms (HIV / HBV).



4.7.3 In all cases, these **Universal Precautions** and General Safety Rules shall be followed:

4.7.3.1 All supervisors must ensure that their staff is trained in proper work practices, the concept of **Universal Precautions**, personal protective equipment, and proper cleanup and disposal techniques.

4.7.3.2 Gloves must be made of appropriate disposable material, usually intact latex or vinyl. They must be used in the following circumstances:

- When an employee has cuts, abraded skin, chapped hands, dermatitis, or similar conditions.
- When performing any first aid procedures
- When cleaning up after an accident or injury involving blood or other body fluids.

4.7.3.3 Employees must wash their hands immediately, or as soon as possible, after removal of gloves or other personal protective equipment and after hand contact with blood or other potentially infectious materials.

4.7.3.4 Utilize the required personal protective equipment i.e.gloves.

4.7.3.5 Treat all human body fluids and items soiled with human body fluids (blood, semen, vaginal secretions, and saliva) as if contaminated with HIV/HBV.

4.7.3.6 All personal protective equipment and clothing contaminated with human body fluids will be discarded.

4.7.3.7 Any cleanup of blood or body fluid will be presoaked (sprayed on the affected area) with antibacterial/viral solution for 10 minutes before being removed. Gloves will be worn when cleaning blood or body fluids.

4.7.3.8 Any suspected exposure to HIV/HBV by human body fluid contact (via broken skin, human bites, cuts or punctures, etc.) should be reported to a supervisor immediately.

4.8 Emergency Procedures / First Aid

4.8.1 When direct exposure or contact with a potentially infectious material occurs, the following precautions shall be taken:

4.8.1.1 The affected area shall be washed immediately with soap and water. 4.8.1.2 All affected mucous membranes shall be flushed with water. 4.8.1.3 The supervisor shall be informed



immediately.

4.8.1.4 The time, nature, and source of the exposure shall be recorded.

4.8.2 In addition, all workplace control measures and PPE in use at the time of exposure shall be noted.

INJURY AND ILLNESS PREVENTION PROGRAM

Introduction to Our Program

State and federal law, as well as company policy, makes the safety and health of our employees the first consideration in operating our business. Safety and health in our business must be a part of every operation, and every employee's responsibility at all levels. It is the intent of Arroyo Developmental Services to comply with all laws concerning the operation of the business and the health and safety of our employees and the public. To do this, we must constantly be aware of conditions in all work areas that can produce or lead to injuries. No employee is required to work at a job or with a consumer known to be unsafe or dangerous to his or her health. Your cooperation in detecting hazards, reporting dangerous conditions and controlling workplace hazards is a condition of employment. Inform your supervisor immediately of any situation beyond your ability or authority to correct. Employees will not be disciplined or suffer any retaliation for reporting a safety violation in good faith.

Safety is a High Priority

The personal safety and health of each employee of Arroyo Developmental Services is of primary importance. Prevention of occupationally induced injuries and illnesses is of such consequence that it will be given precedence over caregiving responsibilities. To the greatest degree possible, management will provide all special training, mechanical and physical protection required for personal safety and health, but our employees must bear primary responsibility for working safely. A little common sense and caution can prevent most accidents from occurring.

Employee Responsibility for Training

Teaching safety is a two-way street. Arroyo Developmental Services can preach safety, but only employees can practice safety. Safety education requires employee participation.

Monthly meetings of all employees will be conducted for the purpose of teaching skills to improve safety. Part of the curriculum will involve applications of the Company's injury and illness prevention program as they relate to actual job assignments. They will discuss general safety rules for specific situations. Remember, the following general rules apply in all situations:



Arroyo

Developmental Services

Emergency Preparedness Policies and Procedures

It is the policy of the ADS program to maintain care and supervision of our consumers in the event of a natural disaster or other large emergency event, until they can safely be released to responsible persons, such as family or caregivers, or first responders such as Fire, Police, Hospital or evacuation shelter personnel when appropriate. To that end, an on-going effort is made to prepare staff and consumers through routine emergency drills; periodic inventory and replenishment of emergency supplies; and administrative review of compliance/areas in need of improvement.

Staff Review of Policies/Training on Procedures

1. **Maintain care and safety of consumers in the event of natural disaster or large emergency event.**
 - A. How long should we continue to care for clients in the event of an emergency?
 - Until they can be safely released to responsible persons, i.e. family and caregivers, first responders, or evacuation shelter.
 - B. An ongoing effort will be made to prepare staff and clients through routine emergency drills.
2. **We are required to conduct annual one-hours in-service training on emergency preparedness.**
 - A. Disaster response plan policy
 - B. Emergency preparedness drills.

Emergency Preparedness Drills. Emergency drills will be performed at minimum every six months, with one drill specifically focused on responding to a major earthquake. Each staff member will be given the "Emergency Drill Preparation" form to guide their group's preparation for a drill. The form contains an ability assessment to rate each consumer's basic understanding of how to respond to a particular emergency type, and their ability to demonstrate how to get to a protected position. On the back of the form is "Responding to Emergencies: Some Basic Rules", that describes initial protective actions to take in the event of fire, earthquake, car accident or breakdown (the "Show Me" section).

For example, when staff prepare to do an earthquake drill, they will discuss with their consumers the "rules" for responding to an earthquake (e.g., DROP/COVER/HOLD ON). Staff will demonstrate DROP/COVER/HOLD ON, and ask each consumer to also rehearse DROP/COVER/HOLD ON. Research has shown that rehearsal of the skills needed to protect oneself makes it more likely that you will use those skills in an actual emergency. Adaptations for mobility-restricted individuals will be reviewed as needed.

On the "Emergency Drill Preparation" form, staff will rate each consumer's ability to understand the "rules" discussed, as well as their ability to verbalize and/or demonstrate what protective skills are called for in the event of an earthquake. Consumers will also be rated on their ability to locate their personal "emergency pack" (see below, under Emergency Supplies) or I.D./contact information placed in their wallet and hand it to someone who has asked for their emergency contacts. This information will be reviewed by ADS administrative staff to identify which consumers may require additional support or training to respond effectively to an emergency.

Staff will perform an emergency drill and complete an "Emergency Drill" form to document the results of the drill. The form also asks that they inventory the emergency supplies in their car and each consumers' personal supplies, in order to identify items missing or in need of replacement. ADS administrative staff will review the drill report to identify which staff or consumers need supplies, and make arrangements to obtain those supplies as soon as possible.

3. Emergency Preparedness Drills --

- A. Should be performed at a minimum every six months—one drill must be an earthquake drill
- B. Emergency Drill Preparation – is used to guide each group's preparation for a drill and assesses each client's basic understanding of how to respond to a particular emergency type. This also contains some basic rules under the "Responding to Emergencies: Some Basic Rules"
 - Discuss rules for the given emergency.
 - staff demonstrate the rules (i.e. DROP/COVER/HOLD on)
 - Participants rehearse the rules.
 - Staff will rate each individual's ability to understand the "rules" as well as their ability to verbalize and demonstrate the rules.
 - Participants will also be rated on their ability to locate their personal "emergency pack"
- C. What is in the client emergency pack? Optional
 - Refillable water bottle
 - Lunch/and or snacks
 - Personal entertainment items (magazines, radio, cell phones)
 - Hygiene items as appropriate (diapers, wipes, sanitary napkins)
 - Additional clothing as appropriate.
 - Copy of face sheet/Medical consent in zip lock
 - One Day of medication
- D. Staff emergency pack
 - What is in the Staff emergency Pack
 - Fully charged cell phone
 - ADS business cards
 - Facesheets/medical consent
 - Other supplies

- First aid kits
- Minimum of one gallon of water
- Emergency snacks
- Battery powered flashlight
- Fire extinguisher

Emergency Locations. Given that ADS services are community based, staff will use available resources to seek shelter, supplies and support while they determine how best to safely transport consumers to their care providers at home or (if unsafe to do so) to a local evacuation center. Community centers, senior centers, city halls, libraries, Police and Fire stations, churches and schools, shopping mall security stations, transit centers, etc., are locations that are prevalent, familiar to staff and consumers, and are likely to contain trained personnel (often with radio contact to first responders) that can share information and resources.

4. Emergency locations

- A. Use available resources to seek shelter, supplies and support while determining how to safely transport clients to their care providers or to local Emergency Operation Centers (EOC)
 - Libraries, police departments, fire stations, community centers, senior centers, city halls, churches and schools, security stations, and transit centers are a few of the resources that contain trained personnel that can be used for additional information.
 - Emergency Operation Centers (EOC)
 - Activated in West Covina when an incident off us I west Covina may be expected to require outside resources for mitigation
 - An incident has occurred outside of our jurisdiction that bears monitoring in anticipation of affecting our city.
 - What is the function of the EOC?
 - Provides support for field operations by locating and allocating additional resources and by anticipating future needs as an incident unfolds while maintaining a continual "situation status". Provides a formal communication link between the community and emergency operations on other communities. EOC began the documentation, and coordination necessary to insure the recovery of eligible funding reimbursement from the state FEMA.

Practical tips for responding to earthquakes or other Major emergencies while at work.

1. Before an Emergency Event Occurs.

- a. Who does the client stay with if the primary caregiver is not available?
- b. Groups homes should identify an evacuation site if the home is seriously damaged. Ask the group home to provide you with this information.

2. Assess the Emergency Event

- a. Check the new frequently

- b. If aware of an event, determine how close and how severe the situation is
- c. If you have yet to report to work, and suspect that there is an event close by, do not pick up clients until you have more information to do so.
- d. Contact the Program Director or Supervisors for more info.

3. Clients in your care during an emergency

- a. Direct clients to use the protective skills learned and practiced during emergency drills. Check for injuries, seek community support
- b. If there is an emergency and able to transport the client safely, go to the nearest emergency room
- c. If it is safer to stay off of the streets, call the caregiver directly and arrange for the client to be taken home. Contact the program director or a supervisor to report the status.

4. There are four types of emergency drills covered on the emergency drill preparation sheet. This can be broken down into "what can clients do?" and "What can Job Coaches do"

A. Fire

- If you hear a smoke alarm, smell fire/smoke, exit the building ASAP
- If there is a lot of smoke and heat what should you do?
 - Cover your face with a wet shirt and crawl to the exit.
- How should you touch a door to feel for heat before opening it?
 - With the back of the hand because there could be a fire on the other side and it will burn your skin.
- What should you do if your clothes catch fire?
 - STOP – DROP – ROLL – to put out the flames
- What should you look for as you are exiting the building?
 - Fire extinguisher

B. Quake

- What should you do in the event of an EARTHQUAKE
 - DROP—COVER—HOLD ON.
- What should you cover in an earthquake?
 - Your head with your arms
- When is it safe to get up and check the area?
 - When the earthquake is over.
- If you are outside, what should you do?
 - Move away from buildings, powerlines, trees, and sit down to avoid falling.
- Check for injuries and provide first aid once you're in a safe location.
- What should you keep in your car in the event of an earthquake or emergency
 - Water bottles, first aid kit, non-cook foods (Granola Bars), battery powered radio.
- When should you call 911 after an Earthquake?
 - When someone is seriously injured and requires medical attention. Remember that emergency service vehicles are going to be very busy after an earthquake.

C. Car-break down

- If your car breaks down on the road what should you do?
 - Stay safe in the car with the seatbelt fastened, until the driver says it is safe to exit.
- When exiting the car on the road, what should you do?
 - Move away from the street and move traffic.
- Who should you call when your car is broken down?
 - Road side assistance and ADS office once everyone is safely out of the car.

D. Car Accident

- First thing that you do if you are in a car accident?
 - Check for injuries, if injured in the car, wait for paramedics to come and help.
- Should you help an injured person outside of the vehicle?
 - DON'T MOVE AN INJURED PERSON UNLESS THEY'RE IN DANGER STAYING IN THE CAR.
 - What can be a danger in the car?
- a. Fire, smell of gas,
- Call 911 or get the attention of others and tell them to call.
- What should you do with an injured person in a car?
 - Assist injured person to stop bleeding/keep breathing until help arrives.

Consumer daypack/tote bag. ADS routinely recommends that consumers and their care providers prepare a personal daypack, tote bag or large purse that they bring to a program that contains a refillable water bottle, their lunch and/or snacks, and personal entertainment items (magazines, radio, cell phone, etc.). Hygiene items (diapers, wipes, sanitary napkins) and additional clothing items are included as appropriate to consumer needs.

ADS staff will place a folded copy of the consumer's face sheet/consent for medical treatment inside of a zip-lock plastic bag ("emergency pack"). Care providers will be asked to provide one day's worth of medications preferably in pharmacy labeled childproof containers for program use in case of emergency. Such medications would ideally fit into the "emergency pack" along with the face sheet/consent for medical treatment, and be secured inside of a daypack/tote bag zippered pocket.

Consumers are encouraged to carry their own daypack/tote bag with them as much as possible, especially when using public transportation. If a consumer repeatedly refuses to use a daypack/tote bag, or there are good reasons not to do so (for example, a tendency to lose things) the "emergency pack" may be carried by the Job Coach in their daypack.

Capable consumers may opt to carry I.D., medical information and emergency contact numbers on a card in their wallet, rather than carry a face sheet. Those who carry a personal cell phone will be encouraged to enter their own "ICE" (In Case of Emergency) contact phone number, as well as phone numbers for the ADS office, their Job Coach's work cell number, and other emergency contacts.

If the request for one day's worth of prescribed medications is repeatedly ignored or formally refused by the consumer's care provider, this will be documented in a letter and sent to the service coordinator within 60 days of the initial request by the program.

Staff daypack/tote bag. Staff are required to carry a fully-charged program issued cell phone, ADS office business cards and a duplicate "emergency pack" containing face sheets for assigned and substitute consumers, as well as consumer medications (when necessary) in their personal daypack, tote bag or purse while they are involved in program activities in the community.

Emergency Supplies/Equipment. Each ADS staff assigned to provide consumer care is required to maintain basic emergency supplies that are kept in the staff vehicle, as well as carried on the person of each member of a program group. These supplies are intended to meet the immediate needs of consumers and staff during the first 24 hours after a major disaster while an effort is made to transport consumers home or (if determined unsafe to do so) to a local evacuation center, where additional supplies and support can be accessed.

Other supplies. Staff are required to keep in their passenger car trunk the following supplies: a first aid kit, a minimum of one gallon of drinking water, emergency snack foods (energy/granola bars), a battery powered flashlight, auto fire extinguisher, and a local map. These supplies are purchased by the program, and will be inventoried by administrative staff during vehicle checks twice a year and by direct care staff in preparation for an emergency drill twice a year.

Communication with Care Providers. ADS staff are responsible to contact their assigned consumers' care providers or other designated emergency contacts directly by cell phone as soon as possible after a major emergency to coordinate how best to return the consumer to their care. For emergency purposes ADS staff may provide their work cell phone number to care providers, and program the work cell phone with care provider cell phone numbers for efficiency. (When phone service is overloaded or otherwise disrupted, text messaging sometimes can connect, which is the reason cell numbers are preferable in an emergency to landline phone numbers.) The "Disaster Response Plan" policy will be given to care providers immediately after ADS conducts its annual in-service training on the subject.



SERVING PEOPLE WITH AUTISM SPECTRUM DISORDER

What is Autism?

Excerpt taken from NEUROCOSMOPOLITANISM; NICK WALKER'S NOTES ON NEURODIVERSITY, AUTISM, AND COGNITIVE LIBERTY

Autism is a **genetically-based human neurological variant**. The complex set of interrelated characteristics that distinguish autistic neurology from non-autistic neurology is not fully understood, but current evidence indicates that the central distinction is that **autistic brains are characterized by particularly high levels of synaptic (neural) connectivity and responsiveness**. This tends to make the autistic individual's **subjective experience more intense and chaotic** than that of non-autistic individuals. On both the sensorimotor and cognitive levels, the autistic mind tends to register more information, and the impact of each bit of information tends to be both stronger and less predictable.

Autism is a developmental phenomenon, meaning that **it begins *in utero*** and has a pervasive influence on development, on multiple levels, throughout the lifespan. **Autism produces distinctive, atypical ways of thinking, moving, interaction, and sensory and cognitive processing**. One way of looking at it is that autistic individuals have a **different neurological "operating system"** than non-autistic individuals.

According to current estimates, **3.2% of the world's population is autistic (1 in 31 children)**. While the number of individuals diagnosed as autistic has increased continually over the past few decades, evidence suggests that this **increase in diagnosis is the result of increased public and professional awareness**, rather than an actual increase in the prevalence of autism.

Despite underlying neurological similarity, **autistic individuals are vastly different from one another**. Some autistic individuals exhibit exceptional cognitive talents and some are intellectually disabled and need constant support. However, in the context of a society designed around the sensory, cognitive, developmental, and social needs of non-autistic individuals, autistic individuals are almost always disabled to some degree – sometimes quite obviously, and sometimes more subtly.

This paper will discuss these conduct issues and provide suggestions for Arroyo Job Coaches on how to respond to and support people with ASD.

Common Symptoms of ASD in Adults Include:

- Difficulty interpreting what others are thinking or feeling
- Trouble interpreting facial expressions, body language, or social cues
- Difficulty regulating emotion
- Trouble keeping up a conversation
- Inflection that does not reflect feelings
- Difficulty maintaining the natural give-and-take of a conversation; prone to monologues on a favorite subject
- Tendency to engage in repetitive or routine behaviors
- Only participates in a restricted range of activities
- Strict consistency to having thing in a certain order and following daily routines; outbursts when changes occur
- Deep knowledge of one particular topic, such as a certain branch of science or industry

Social Isolation. People with ASD tend to be socially isolated and do not quickly and easily form bonds with new people in their lives. They do not generally like to be touched, to have people too close, or to have a lot of talk and conversation. They may not look directly at people who address them. There can be a lot of anxiety, stress and agitation expressed during the initial meetings and first weeks of supporting them. This is often when extreme forms of self-injurious behavior, aggression and destruction occur. They may also attempt to escape new social demands by eloping. Once routines and social bonds form, they often show much more tolerance and even expect certain people to always be in their lives.

Getting To Know Someone With Autism. New Job Coaches should accompany people the person knows and likes if possible and introduce themselves calmly and gradually. They may drop by the home for short visits before the person actually starts the program; they can eat lunch or spend time with an existing group before taking over, etc. If this is not possible, then the Job Coach should read and learn from people who know the person before trying to support them alone. They should speak directly to the person from a short distance and offer a hand to introduce themselves. They should avoid getting too close, avoid touching them (even with affection), and allow them to offer a hand in return (or not). The Job Coach should not say too much, or talk about the person in the presence of the person.

Going Out into the Community. The Job Coach should be very concrete with directions and about plans for the day. They should not ask too much of the person with ASD at

first and try to go along with their routines and compulsions. Initially, seating them in the car should ideally provide for personal space around them. They may be very intolerant of many common locations in the community, and initial settings visited should not be crowded, loud, or sensitive to conduct issues. Places with food and drinks may need to be avoided until their conduct around these items is known. They should not be taken to stores with items they cannot afford, as they may insist on purchasing something before they are willing to leave. Large parks are the safest place to start to build a relationship and long walks help channel nervous energy while people adjust to each other.

Transitions. People with ASD can experience stress and anxiety during the transition from one program to another, from one activity to another and the transfer from one staff to another. Conduct issues may surface during pick-ups and drop-offs, during the end of one activity and the start of another, and during changes in shifts. Not informing them in advance, hurrying them, speaking harshly to them (or around them) makes the situation worse. Transition conduct may diminish over time as the person becomes tolerant of routines, but some people may need a lot of support each day to enter and exit the program. The Job Coach should consider how they could lower the stress level of each transition. This may involve speaking to them in advance, allowing extra time, offering special incentives once the transition is completed (money, snacks, drinks, special trips) and if the person is too agitated, going away and returning to try again later. Other people who make the transitions stressful may need to be educated or avoided.

Communication. People with ASD have unusual ways of expressing themselves as well as difficulty perceiving and understanding complicated social interactions. Many conduct issues have been attributed to their difficulty saying what they want, identifying how they feel, and learning how to respond to and negotiate with other's demands and needs without conduct issues. Some are aphasic (no speech, but they may have "happy" and "anxious" sounds), some echo some of the speech they hear (directly or from the past), some talk excessively and in great detail about topics they obsess on mostly. They may have devices that speak for them, bring and use pictures or symbols to represent things they want, and/or have a repertoire of conduct issues that effectively get them what they want.

The Job Coach will listen to them, encourage them to "use their words", and help make and/or use communication methods that were effective in their past. The Job Coach will make every effort to learn how they communicate and what they are saying and offer prompt and honest responses to each effort to communicate. This may lead to providing what they ask for right away or in the near future plans of the group. Even if the response to the request is a form of refusal ("maybe later", "we already talked about that", "let's ask your home", etc.) acknowledging each effort to communicate diffuses

anxiety over not being heard. Staff insights about the content of each person's communication and how to get them to communicate more should be noted and shared with other people trying to support the person.

Routines and Responding to Change. Some people with ASD find great comfort in closely following routines to the point of ritualistic adherence to a certain order of things. Others prefer to be told about changes before they occur and need time and lowered expectations when moving through them. Once the Arroyo program and a specific Job Coach becomes part of the expected routine of a person with ASD, they often show far less conduct issues, tolerate more, and form strong social bonds. The Job Coach must show consideration to the person they support with ASD about following routines and provide extra attention when there are changes in plans.

Obsessions and Compulsions. People with ASD often have things that they are very focused on. This can be topics of interest, the order of things, drinking certain drinks and/or eating certain foods, the collecting and holding of specific objects, even how they move through space (two steps forward, one step back). It may include how they eat, drink, wash their hands, toilet, how they behave around water, animals, children, etc. They may not tolerate sneezes, florescent lights, or certain sounds. The conduct surrounding these obsessions and compulsions are well developed, often tied to feelings of anxiety, and is well described in their history.

The Job Coach needs to know what the conduct consists of, how it has been accommodated (or controlled) in the past and how they are going to respond to it according to the Arroyo behavior plan. This may involve avoidance of certain places and people, the increase of support when certain things are happening, and the careful counseling of the person on what the rules are regarding their conduct in this area.

Over-stimulation. People with ASD often get over-stimulated by sound, light, crowds, touch, too much speech, seeing others experience anxiety, etc. The Job Coach must be aware of what specifically bothers the people they support and the signs that they are not coping well. For example, when a person puts his fingers in his ears he is trying to block sounds that are bothering him. The Job Coach may provide sound-canceling earphones, may leave the setting, or just monitor the person depending upon the potential for conduct issues.

Self-Injurious Behavior (SIB). Some people with ASD express anxiety with highly practiced forms of self-injury. They may have calluses built up from a lifetime of specific bites, hits, banging, etc. Their history of SIB is longstanding and often functions to express and control anxiety, to generate a specific response from caregivers, and to force predictable changes in demands, settings, etc. The Job Coach should learn about the history of the SIB, what situations are likely to trigger it, and what the behavior plan

for responding to it is. Often, no comment, intervention, or consequence is directed to the SIB conduct when it occurs. Instead, there will be an effort to gauge and respond to the anxiety that triggered it.

Tolerance and Anxiety. People with ASD have high levels of anxiety and as a result show intolerance of many settings and circumstances similar to where they have had negative past experiences. They need help managing frequent anxiety with proven strategies provided by their Job Coach or else they will use established maladaptive conduct to express and manage it themselves. Proven strategies are provided in behavior plans and often include the following features:

- Effective communication: from the person with ASD and to them by their caregivers. "PLEASE USE YOUR WORDS!" This gives people warning about what is about to happen, provides comfort and coping benefits and provides a functional alternative to severe conduct.
- Relaxation strategies that include deep breathing, music, snacking, moving away, pacing, rocking, etc. should be established and practiced before needed so they can be called upon when the person is stressed.
- Self-control and self-direction strategies need to be in place where the person with ASD needs only request the support they require to get a reliable response from their Job Coach. Saying for example "I need to go!" could lead to the group moving to another place to escape the triggering setting. "I need to talk" could lead to one-on-one support from the Job Coach.
- Changing the setting, allowing them to stop participation, can often allow the person experiencing anxiety to escape the circumstances provoking them. It can be as simple as going to the bathroom or as major as packing up and leaving the setting.
- Offering people options, incorporating their known interests in daily plans and giving them control over their participation in activities is proven to lessen anxiety and build tolerance.
- Tolerating self-stimming, SIB, acting on obsessions and compulsions and mild forms of target behaviors (but not reinforcing them with attention) may help a person manage their anxiety. They may need to be directed to a setting where they will keep their dignity. This can be tolerated as long as the conduct is not extreme, does not cause harm and is used as a half measure while the person learns more appropriate ways to manage anxiety.
- Providing incentives for good conduct (often defined as the absence of targeted behaviors) provides motivation for self-control. The closer the incentive matches something they care about, the better it will work. How often, the amount offered, and the circumstances where it is provided or denied are specified in a behavior

